



Armada Kalamunda Group

Dysphagia Management for Emergency Department Allied Health Team Guideline

1. Purpose

To guide Emergency Department Allied Health Team (ED AHT) in the identification of patients with, or at risk of dysphagia, the process of screening the patient and method of referring to Speech Pathology (SP).

The RBWH (Royal Brisbane Women's Hospital) Dysphagia Screening Tool (DST) AKMR140.2 is administered to Emergency Department patients identified as being at risk of aspiration by trained Emergency Department AHT members

2. Applicability

This guideline applies to the Occupational Therapy, Physiotherapy and Speech Pathology Disciplines within the Emergency Department of Armadale Health Service.

3. Guidelines

As many as 15-30% of patients in hospital have evidence of swallow difficulties.

Identifying swallowing problems early may prevent complications such as:

- Malnutrition
- Dehydration
- Increased risk of chest infections and aspiration pneumonia
- Longer length of stay
- Discharge to nursing home
- Higher mortality

There is currently no SP cover over the weekend for AKG sites requiring alternative options for managing dysphagic patients.

3.1 Assessment

- The ED AHT to ask the patient questions regarding their swallow function such as: "Do you find that certain foods, drinks or medications are difficult to chew, get stuck in your throat or go down the wrong way and make you cough?"
- If patient answer yes, complete **RBWH Dysphagia Screening Tool** (AKMR A234): [Appendix1](#) if trained to do so
- If not trained to complete Dysphagia Screening tool contact trained staff to complete the screen –other ED AHT member or ward staff.
- Contact Speech Pathology on 9391 2219 or 9391 2395 to organise training as required.

Before referencing this guideline document please ensure that you have the latest version from the [Policy Library](#) information hub page.

3.2 Referral

3.2.1 FOR PATIENTS TO BE ADMITTED TO INPATIENTS

(Including assessment within ED Mon – Friday 8.30 -3.30)

- Ensure need for SP is highlighted on ward transfer documentation and complete iCM handover
- Contact SP dept on ext 2219 or 2395, pg. 602/ 159/ 219
- Provide patients details, including:
 - Name
 - UMRN
 - Diagnosis
 - Reason for referral
 - Bed number

Speech Pathology will be responsible for referring patients on to Outpatients (OP) post their inpatient admission if appropriate via e-referrals

3.2.2 FOR PATIENTS NOT ADMITTED BUT REQUIRE OUTPATIENT REVIEW

- Refer to Speech Pathology outpatients via WA Health e-Referrals

4. Legislative context

The following documents are required to give effect to this policy [i.e. the documents included are mandatory]:

- Health Department of Western Australia (HDWA) [Clinical Handover Policy MP0095](#)
- East Metropolitan Health Service (EMHS) [Consent Policy EMHS: 5](#)
- EMHS [Patient Identification and Procedure Matching Policy EMHS: 21](#)

5. Supporting information

The following documents support the implementation of this policy:

- [Dysphagia Screening and Assessment Procedure AKG-PRO-0140-14](#)

6. Compliance, monitoring and evaluation

Compliance against this guideline will be evaluated with annual medical file audits.

7. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 2 – Action 2.4: The health service organisation ensures that its informed consent processes comply with legislation and best practice
- Standard 5 – Action 5.10: Clinicians use relevant screening processes:
 - a. On presentation, during clinical examination and history taking, and when required during care
 - b. To identify cognitive, behavioural, mental and physical conditions, issues, and risks of harm

- c. To identify social and other circumstances that may compound these risks
- Standard 5 – Action 5.11: Clinicians comprehensively assess the conditions and risks identified through the screening process
- Standard 5 – Action 5.12: Clinicians document the findings of the screening and clinical assessment processes, including any relevant alerts, in the healthcare record
- Standard 5 – Action 5.27: The health service organisation that admits patients overnight has systems for the preparation and distribution of food and fluids that include nutrition care plans based on current evidence and best practice
- Standard 5 – Action 5.28: The workforce uses the systems for preparation and distribution of food and fluids to:
 - a. Meet patients' nutritional needs and requirements
 - b. Monitor the nutritional care of patients at risk
 - c. Identify, and provide access to, nutritional support for patients who cannot meet their nutritional requirements with food alone
 - d. Support patients who require assistance with eating and drinking
- Standard 6 – Action 6.3: Clinicians use organisational processes from the Partnering with Consumers Standard to effectively communicate with patients, carers and families during high-risk situations to:
 - a. Actively involve patients in their own care
 - b. Meet the patient's information needs
 - c. Share decision-making

8. Glossary

The following definitions are relevant to this policy.

Term	Definition
Swallowing	The movement of a bolus of food, fluid or saliva from the mouth to the stomach.
Dysphagia	A disorder or a symptom that can be caused by structural, physiological and /or neurological impairments affecting the preparatory, oral, pharyngeal, and/or oesophageal stages and can result from a wide variety of medical conditions.
Aspiration pneumonia	Bronchopneumonia resulting from inhaling of foreign material into the bronchi.

9. Document owner

Coordinator Speech Pathology

Enquiries relating to this guideline may be directed to:

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10. Review

This guideline will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V3	15/11/2024	15/11/2027	3.1 Assessment – Added in the other Speech Pathology contact phone number. 3.3 and 3.4 – Amended numbering as incorrect. 3.2.2 Removed ‘single point of referral allied health and community rehab form’ and replaced with WA Health e-Referrals as an avenue to refer to outpatient services

11. Authorisation

This guideline has been authorised and issued by the Director Allied Health and Ambulatory Care.

Authorised by	Danielle Kilmurray – Director Allied Health and Ambulatory Care
Authorisation date	14/11/2024
Published date	15/11/2024

Appendix 1: RBWH Dysphagia Screen Tool

Please use I.D. Label or BLOCK PRINT

ARMADALE HEALTH SERVICE RBWH DYSPHAGIA SCREENING TOOL WARD _____ DOCTOR _____	Family Name: _____ UMRN: _____ Given Names: _____ Gender: _____ DOB: _____
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Screening Procedure for Patients with Dysphagia and / or Aspiration Risk
Only to be used by trained clinical staff

PHASE 1
Does the patient have any of the following or previous history of the following? (tick if applicable)

☐ Yes / ☐ No	Dysphagia or aspiration risk on previous admission(s)
☐ Yes / ☐ No	COPD +/- upper GI disorder
☐ Yes / ☐ No	Stroke
☐ Yes / ☐ No	Neurological involvement
☐ Yes / ☐ No	Head injury
☐ Yes / ☐ No	Head & neck (H&N) cancer +/- H&N surgery / chemotherapy / radiation
☐ Yes / ☐ No	Orthopaedic conditions: # NOF, cervical spinal surgery
☐ Yes / ☐ No	Acutely unwell AND frail aged with co-morbidity
☐ Yes / ☐ No	Suspected aspiration pneumonia / recurrent chest infections
☐ Yes / ☐ No	Severe disability (e.g. physical disability)

– If 'Yes' to any of the above boxes proceed to **Phase 2**

– If all boxes are ticked 'No', patient to have usual modified diet or general diet & thin fluids.

– Complete 'Swallowing Management Plan' below

PHASE 2
Interview with Patient / Family / Carer (tick if applicable or assessable)

☐ Yes / ☐ No	Do you have difficulty eating food / drinking fluid?
☐ Yes / ☐ No	Do you cough or have choking episodes whilst eating / drinking?
☐ Yes / ☐ No	Do you need to modify the food you eat? e.g. softer or smaller pieces
If yes, provide details of current diet / fluid consistency: Diet _____ Fluid _____	
☐ Yes / ☐ No	Do you get chest infections? Is this recent?
☐ Yes / ☐ No	Has this been investigated before?

Indicators of Dysphagia / Aspiration Risk (tick if applicable)

☐ Yes / ☐ No	Altered LOC / reduced responsiveness
☐ Yes / ☐ No	Respiratory rate >30bpm
☐ Yes / ☐ No	Slurred speech (dysarthria)
☐ Yes / ☐ No	Weak / absent volitional cough
☐ Yes / ☐ No	Not managing oral secretions (drooling, wet voice)
☐ Yes / ☐ No	Weak voice (dysphonia)
☐ Yes / ☐ No	Suspected aspiration pneumonia / recurrent chest infections
☐ Yes / ☐ No	Documented or history of dysphagia / aspiration risk
☐ Yes / ☐ No	Reports difficulty swallowing
☐ Yes / ☐ No	Reported coughing / choking episodes whilst eating / drinking

Water Test: Administer only if 'no' has been ticked for all indicators of dysphagia. Give the patient 85mls (1/3 cup) water

☐ Yes / ☐ No	Coughing during or between swallows, or up to one minute after swallowing
☐ Yes / ☐ No	Wet / gurgly or hoarse voice after swallowing
☐ Yes / ☐ No	Increased respiratory rate after swallowing

– If 'Yes' to any of the above **Indicators** or the **Water Test**, refer patient to speech pathologist. Patient should remain NBM until speech pathologist review in consultation with the medical team. If a speech pathologist is unavailable, consider nutrition / hydration options such as giving IV fluids / NG feeds under medical supervision/advice.

– If 'No' to all of the above **Indicators** and the **Water Test**, place patient on usual modified diet or general diet & thin fluids

SWALLOWING MANAGEMENT PLAN

☐ Yes / ☐ No	Patient identified as dysphagic or aspiration risk
If 'Yes':	
Recommended patient remain NBM	☐ Yes / ☐ No IV fluids ☐ Yes / ☐ No NG feeds ☐ Yes / ☐ No
Medical officer notified	☐ Yes / ☐ No Date referred: ____/____/____
Speech pathologist notified	☐ Yes / ☐ No Date referred: ____/____/____
If 'No':	
☐ Patient placed on general diet and thin fluids; or	
☐ Patient placed on usual modified diet / fluids: Diet _____ Fluids _____	

Print Name _____	Signature _____	Designation _____
Ward _____	Unit _____	Date _____ Time _____

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AKMR140.2 RBWH DYSPHAGIA SCREENING TOOL

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